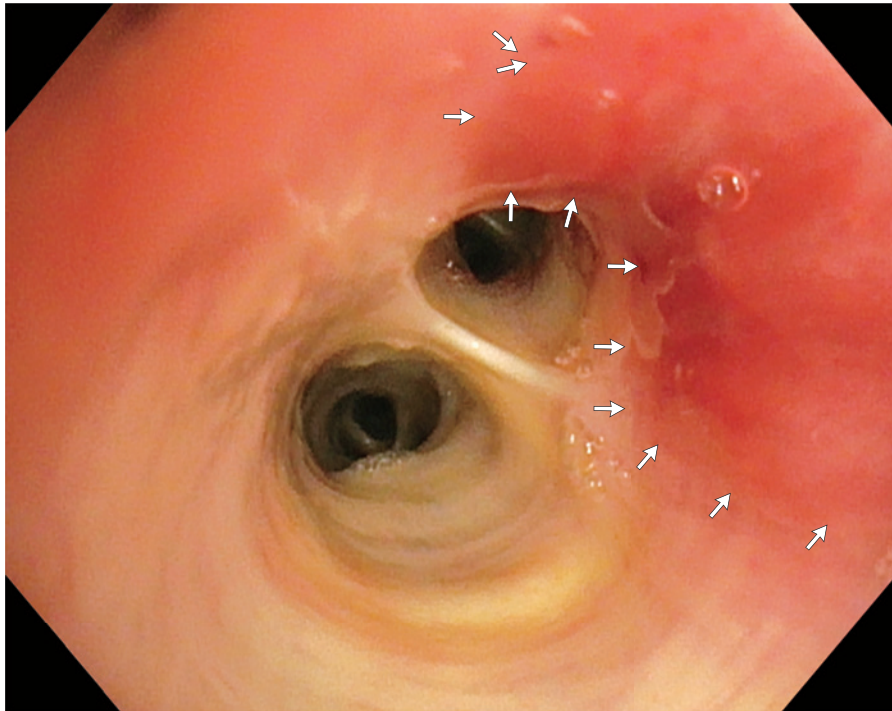


IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Endobronchial Kaposi's Sarcoma



A 55-YEAR-OLD MAN WITH A HISTORY OF ORTHOTOPIC LIVER TRANSPLANTATION complicated by post-transplantation Kaposi's sarcoma was noted during bronchoscopy to have a darkened patch of endobronchial mucosa (arrows) in a segmental bronchus of the left lower lobe. One year before the current presentation, the patient had received a diagnosis of post-transplantation Kaposi's sarcoma of the liver, ventricle, and lymph nodes. Treatment had been started and resulted in a good response. Surveillance computed tomography of the chest that was performed several days before the current presentation had shown new pulmonary opacities, for which the bronchoscopy had been ordered. The patient had reported no respiratory symptoms. Histopathological analysis of a biopsy specimen of the lesion showed spindle-shaped cells forming vascular structures. The cells stained positive for human herpes virus 8 on immunohistochemical analysis. A diagnosis of endobronchial Kaposi's sarcoma was made. Recipients of solid-organ transplants are at risk for post-transplantation Kaposi's sarcoma owing to the use of immunosuppressive drugs. Endobronchial involvement may be asymptomatic — as in this case — or result in cough or hemoptysis. Transbronchial biopsy of the lung showed organizing pneumonia, which was probably induced by everolimus toxicity, as opposed to Kaposi's sarcoma. Treatment for Kaposi's sarcoma was continued without modification, except for tapering of the everolimus dose.

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